



STRATEGY & OPERATIONS

Vytalix Funding Strategy

Vytalix company-build documentation · reference
09_FUNDING_STRATEGY

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR
The Founder, Vytalix

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Owner: Fundraising Director · **Status:** PROPOSED v1 (8 September 2026) · **Governing file:** /FACTS_BASE.md · **Numbers from:** /docs/08_FINANCIAL_MODEL.md (scenario model v1.1) · **Structure from:** /docs/02_GROUP_STRUCTURE.md · **Investor assets:** /docs/10_INVESTOR_PROPOSITION.md , /assets/INVESTOR_ONE_PAGER.md , /assets/INVESTOR_PITCH_DECK.md , /investors/

Truth standard. Vytalix is pre-formation: no entity, no revenue, no customers, no grants, no investors, no documented ownership of MaternaLink (FACTS_BASE §1). Nothing in this document is a record of money raised or promised. Every round, valuation, percentage and date is PROPOSED or ESTIMATE. All SEIS/EIS limits, reliefs and tax figures are **TO VALIDATE** with an accountant and against gov.uk before any use with an investor. GBP throughout. Nothing here is legal, tax or investment advice.

Executive view

- The money follows the milestones, not the calendar.** Vytalix should raise three staged rounds — Pre-seed £250k–£500k (2027), Seed £1.5m–£3m (2028), Series A £6m–£12m (a 2029–30 decision) — and each is opened only when a short list of facts is TRUE (entity, IP, safety artefacts, design partner, paid pilot, evidence). Raising before the facts are true burns the best investors in a small pool.
- Services and grants come first, and equity is sized from the model, not from ambition.** 08_FINANCIAL_MODEL shows a peak cash need of £229k (Conservative), £1.06m (Base) and £145k (High-growth) over 36 months, with months 2–5 the solvency pinch-point in every scenario. The Base pre-seed of £400k in month 4 covers the product-lead hire in month 7; the £2.0m seed in month 20 funds MaternaLink's build, evidence and team. Grants (£75k–£400k in the model) are upside, never load-bearing.
- SEIS/EIS is the pre-seed engine.** UK angels expect relief; advance assurance must be applied for within weeks of incorporation (commonly cited limits: SEIS £250k per company and £200k per investor per year; EIS £12m lifetime — all TO VALIDATE). Because the Base pre-seed exceeds the SEIS cap, it is structured as SEIS first (£250k) then EIS (£150k), and consultancy-heavy trading substance is the eligibility risk to design around.
- Dilution is planned in writing before the first conversation.** Illustrative cap table: founder 100% → ~79% after the MaternaLink IP settlement and option pool → ~68% after pre-seed → ~51% after seed → ~37% after Series A, with the IP counterparty vesting from ~10% to ~4%, a 12% pool, advisors inside the pool, and investors holding ~14% / 25% / 29% per round (ESTIMATE; pre-money placeholders £2.5m / £6m / £20m TO VALIDATE against comparable rounds).
- Four raise-killers must be closed before any investor is called:** undocumented MaternaLink IP, unvalidated clinical claims from the 2025 deck, no legal entity, and the Ekiti pricing (£300/woman/year, £4.5m–£36m phases). Each is fixable in 30–60 days; none is fixable after an investor has found it in diligence.

1. Where the money can come from — assessment of every route

Scoring: Fit (does it suit a pre-formation UK health-tech with a services cash layer and a maternal-health product in development?), Timing (when it is realistically available), Cost (dilution, time, conditions), Verdict.

#	Route	What it is for Vytalix	Fit	Timing	Cost / conditions	Verdict
1	Bootstrapping (founder cash + services revenue)	Opening cash £25k–£50k (founder + director's loan, TO VALIDATE) plus Advisory/Consulting invoices from month 2. Base services revenue £166k in FY2027 (08 §3.2).	Very high — it is the only money available today and it proves demand	Now	Founder time (the binding constraint); no dilution; services-only ceiling (Conservative scenario)	Primary until pre-seed closes; permanent cash layer thereafter
2	Angels (SEIS/EIS)	£25k–£100k cheques from health-tech, women's-health, NHS-experienced and diaspora angels; syndicates (GC Angels, NorthInvest and others in /investors/TOP_25_INVESTORS.md)	High for pre-seed; angels accept pre-revenue if entity, IP and relief are in place	Month 2–6 after incorporation	11–15% dilution for £250k–£500k (ESTIMATE); relief-dependent, so advance assurance is a gating item; 25–40 conversations per close	Lead source for pre-seed

#	Route	What it is for Vytalix	Fit	Timing	Cost / conditions	Verdict
3	Venture capital (pre-seed/seed funds)	UK pre-seed funds (Ada Ventures, Octopus first-cheque, LocalGlobe, AlbionVC health hub, o2h) and Africa-focused funds (TLcom, Ventures Platform, Founders Factory Africa)	Medium at pre-seed (most want a team of 2–3 and a product); high at seed once pilot evidence exists	Seed (2028) realistically; a pre-seed VC lead is possible but not required	20–30% per round; governance rights; expectation of venture-scale outcomes that a services-heavy company must justify	Seed lead; optional pre-seed participant
4	Strategic investment (corporates)	Maternity EPR vendors (System C, K2, Magentus), pharma women's-health units, telecoms/insurers in Nigeria (MTN, Airtel, HMOs), UK benefits platforms	Low now (nothing to be strategic about); medium at Series A	2029+	Channel conflict, exclusivity demands, signalling risk if the strategic passes later	Not before Series A; treat as partner, not investor, until then
5	Grants — Innovate UK Smart	Business-led R&D grants; open competitions (round timing and thresholds TO VALIDATE)	High once incorporated; needs a credible R&D plan (offline-first architecture, safety-by-design)	First bid month 3–6; award 4–6 months later if won	Match funding (typically 30–55% of eligible costs for SMEs — TO VALIDATE); competitive (single-digit to ~20% success, ESTIMATE); bid-writing ~15 founder days	Bid 1 or 2 in year 1
6	Grants — SBRI Healthcare	NHS-themed competitions (100%-funded development contracts, TO VALIDATE current structure); Top-25 investor #1 by fit	High if a competition theme matches maternity, health inequalities or care coordination; the NHS "customer" framing suits MaternaLink v1	Themed calls; check calendar quarterly	Requires an NHS problem statement and, in practice, an NHS clinical sponsor letter; Phase 1 typically ~£100k (ESTIMATE, TO VALIDATE)	Bid when a theme fits; do not force-fit
7	Grants — NIHR i4i (Product Development Award; i4i Connect)	Translational funding for medical technologies	Eligibility caveats: i4i requires an NHS or academic partner and typically funds technology with a clinical claim pathway; Vytalix's v1 is deliberately non-diagnostic, and it has no NHS partner (KNOWN). i4i Connect (SME early-stage) is the only plausible entry and only after a design partner exists (TO VALIDATE current schemes)	Not before month 9–12	Consortium building; ethics/HRA processes; reporting burden	Year 2, only with an NHS partner; not in the year-1 plan
8	Grants — Wellcome	Research funding (Discovery Awards, Mental Health, Climate & Health etc.)	Low as a <i>company</i> funder — Wellcome funds researchers and institutions; a route exists only via a university co-applicant for the v3 research track	2028+	Academic lead required; Vytalix as collaborator/subcontractor	Watch; approach via a university partner only

#	Route	What it is for Vytalix	Fit	Timing	Cost / conditions	Verdict
9	Grants — Grand Challenges Canada	Seed and transition-to-scale funding for LMIC health innovations; Top-25 investor #9	Medium–high for the Nigeria evidence programme (maternal, newborn health is a stated priority — TO VALIDATE current calls)	Calls open periodically; first application after a Nigerian implementing partner is signed	Requires an LMIC partner and an evidence plan; CAD-denominated; reporting	Bid in year 2 with a Nigeria partner
10	Grants — Gates Foundation / Grand Challenges	Large, mostly institutional maternal/newborn programmes	Low direct fit for a UK pre-seed company; realistic only as a sub-grantee of an implementing NGO	2028+	Consortium; long cycles	Sub-grantee route only
11	Grants / programmes — MSD for Mothers	Funds maternal-health programmes and social enterprises; funds mDoc's Digital Mom Project in Ekiti and Lagos (VERIFIED)	Medium — and doubly important: the funder of the Ekiti incumbent. A conversation about complementarity (facility-side coordination, DHIS2 export) is worth more than a competing bid	Conversation month 6–9; any bid year 2	Programme framing; partner-first model (05_MARKET_STRATEGY \$2)	Relationship first, bid later
12	Other UK government / public funding	Start Up Loans (British Business Bank, personal loan to founder, TO VALIDATE terms); R&D tax relief (upside, unmodelled); Innovate UK Business Growth (advisory, not cash); NHS Clinical Entrepreneur Programme (network); Health Innovation Network programmes; local growth hubs	Medium; small amounts; mostly credibility and network	Month 1–12	Personal liability (Start Up Loans); administrative	Use selectively; do not depend on
13	Impact investment	Impact funds and DFIs with women's-health or SSA health mandates (Global Innovation Fund, Villgro Africa, GCC transition funds; UK impact angels)	High for the Nigeria/Kenya evidence programme; aligned with patient timelines	Seed and later; some at pre-seed via angel networks	Outcome metrics and reporting; sometimes concessional; sometimes slow	Co-lead at seed; target for pre-seed angels
14	Healthcare / women's-health specialist investors	Goddess Gaia Ventures, Nina Capital, AlbionVC health, Cambridge Enterprise Seed Funds and equivalents	High — they price the thesis correctly and tolerate regulatory timelines	Pre-seed participation; seed lead	Rigorous clinical and regulatory diligence — exactly why the raise-killers must be closed first	Priority list for seed; early relationship-building
15	Revenue-funded growth	Retainers (each ≈ £91k of three-year EBITDA held Y2–Y3), consulting projects, e-learning	High — and the model says it is the best money in the company (08 \$7)	Continuous	Founder capacity ceiling; cannot fund MaternaLink's team (Conservative scenario ends month 36 with £21k)	Permanent; funds the company, not the product build

#	Route	What it is for Vytalix	Fit	Timing	Cost / conditions	Verdict
16	Debt / revenue-based financing / convertibles	Bank debt (unavailable pre-revenue), RBF (needs recurring revenue), ASA/convertible notes as a bridge	ASA (Advance Subscription Agreement) is useful as an SEIS/EIS-compatible bridge if the pre-seed is slow (TO VALIDATE conditions — 6-month longstop, no interest, no redemption)	Bridge only	Discount/valuation cap negotiations; SEIS/EIS compatibility rules	Bridge instrument only

Conclusion. The stack is: **services + founder cash (now) → SEIS/EIS angels pre-seed (2027) with one or two grant bids in flight → impact/health-tech seed (2028) on pilot evidence, with Grand Challenges Canada / SBRI as non-dilutive complements → Series A decision (2029–30) with a strategic as a possible participant.** This is FACTS_BASE A7 with the amounts and gates made explicit.

2. SEIS/EIS — the pre-seed engine (every figure TO VALIDATE)

Item	Commonly cited position (TO VALIDATE against gov.uk / accountant before use)	Implication for Vytalix
SEIS company limit	£250,000 lifetime per company	The Base pre-seed of £400k must be split: SEIS £250k + EIS £150k. Conservative £250k can be all SEIS.
SEIS investor limit	£200,000 per investor per tax year	No single angel can take the whole SEIS tranche; plan for 5–12 SEIS investors.
SEIS company conditions	Trading for under 3 years; gross assets under £350k before the share issue; fewer than 25 full-time-equivalent employees; qualifying trade; UK permanent establishment	Vytalix qualifies on age, assets and headcount at incorporation. Qualifying trade is the risk (next row).
Excluded / scrutinised activities	Certain financial, legal, property and royalty/licence-fee activities are excluded; management consultancy is generally not excluded, but HMRC examines trading substance	Present the application as a technology company with an R&D product programme and a related services trade , not as "a consultancy that hopes to build a product". Keep MaternaLink R&D spend visible in the plan. If the IP route is a licence with royalties rather than assignment, take specific advice on "receiving royalties or licence fees" exclusions.
Income-tax relief	SEIS 50%; EIS 30% of the amount invested (subject to annual caps); CGT exemption on exit after 3 years; loss relief; SEIS reinvestment relief	Explains why UK angels will not invest without advance assurance. Never say "SEIS-approved" — say "advance assurance applied for / received" as true.
EIS limits	£5m per 12 months; £12m lifetime (higher for knowledge-intensive companies — TO VALIDATE thresholds and KIC test); within 7 years of first commercial sale (10 for KIC)	Seed (£2m) and even Series A (£6m–£12m) can be EIS-eligible in part; the 7-year clock starts at first commercial sale, which the services trade may trigger in 2027 — take advice on whether services invoices start the clock.
Share conditions	Full-risk ordinary shares; no preferential rights to assets on a winding-up; no linked loans; money must be spent on the qualifying activity within a set period; no pre-arranged exit	One ordinary share class until the seed (02 \$2.1). Investors' side letters must not create "linked" arrangements.
Advance assurance	Voluntary HMRC opinion; needs a business plan, financials, articles, and named prospective investors (a list of intended investors is now expected — TO VALIDATE current requirement)	Apply in month 1–2 after incorporation with the accountant; expect 4–8 weeks (ESTIMATE). Pipeline conversations can run in parallel provided the status is described truthfully.
Risk-to-capital condition	The company must have growth and development objectives and the investment must carry real risk	Satisfied by the product programme; document it in the plan submitted.

Structure PROPOSED: SEIS tranche closes first (month 3–4), EIS tranche second (month 4–6) — because the SEIS shares must be issued before any EIS shares on the same day or earlier (ordering rule — TO VALIDATE). An ASA is the fallback if closing slips: it keeps SEIS/EIS compatibility only if drafted to HMRC's conditions (no interest, no refund, conversion within six months — TO VALIDATE).

3. Staged rounds — target, use, milestones, investors, thesis, dilution

3.0 How the round sizes tie to the cash need in 08_FINANCIAL_MODEL

Model output (Base unless stated)	Value	Round sizing consequence
Minimum cash before pre-seed	£6k (month 3)	The pre-seed must close by month 4–6 or the founder draw and the product-lead hire (month 7) are deferred. The first 90 days are the solvency pinch-point in every scenario.
Cash need to month 12	~£36k	Services could carry a founder-only company; the pre-seed is buying the product lead, safety artefacts and MVP , not survival.
Cash need to month 24	~£253k	A £400k pre-seed with £166k FY2027 services revenue covers this with margin — hence £400k, not £250k, in Base.
Peak cash need to month 36	£1.06m (still growing ~£65k/month)	The seed is sized at ~2× the need to month 36 (£2.0m) to provide 18–24 months of post-seed runway; closes month 36 with £1.42m (23 months).
Conservative peak need	£229k	A £250k SEIS-only pre-seed survives 36 months only by not hiring a second technical salary — the services-only outcome.
High-growth peak need	£145k	Grants (£400k) and early pilots reduce the equity need; the £3.0m seed in month 14 is a growth decision, not a survival one.
Series A	Not needed for solvency in any scenario within 36 months	A month 30–42 growth-capital decision, sized £6m–£12m to scale to 5–10 UK sites and 2–3 African programmes (ESTIMATE).

Reconciliation note. 02_GROUP_STRUCTURE §2.2 and /assets/BUSINESS_PLAN.md describe a £150k–£300k pre-seed in months 9–15; 08_FINANCIAL_MODEL v1.1 sizes it at £250k–£500k in months 3–6. This document follows the financial model. The earlier figures should be updated to £250k–£500k (with the earliest close in month 4 in Base and the milestone gates below) when those documents are next revised.

3.1 Pre-seed — £250k to £500k (PROPOSED; Base £400k in month 4, i.e. April 2027)

Element	Position
Target	£250k (Conservative, all SEIS) / £400k (Base: £250k SEIS + £150k EIS) / £500k (High-growth). Close in month 3–6 of trading.
Instrument	Ordinary shares at a priced round (preferred by SEIS angels) or an SEIS/EIS-compatible ASA if the lead is slow. One share class.
Valuation (ESTIMATE, TO VALIDATE against comparable rounds)	Pre-money placeholder £2.0m–£3.0m; £2.5m used in the illustrative cap table. 02 §2.2 notes a £1.5m–£4m range for a pre-revenue UK health-tech at this stage.
Use of funds (Base £400k, ESTIMATE)	Product lead/CTO from month 7 (£65k salary + on-costs, ~£55k in-year) · MVP build and penetration test (contractors, ~£90k) · Clinical Safety Officer and clinical advisory group (~£25k) · DPIA, DSPT, Cyber Essentials, ICO, DTAC readiness (~£15k) · Discovery research and co-design in both markets, incl. paid community partners (~£30k) · Legal/IP/trademark completion and SEIS/EIS (~£10k) · Grant match funding reserve (~£30k) · Founder salary top-up to £48k (~£25k of the year) · Working capital and 6-month buffer (~£120k).
Milestones that must be TRUE before opening the raise	1. Vytalix incorporated in England & Wales, bank account open, accountant appointed (VERIFIED absence today). 2. MaternaLink IP assignment deed (or exclusive licence with option) executed ; prototype audit report showing authorship, third-party components and no real patient data. 3. The 2025 predictive/diagnostic claims retired from every external asset; v1 intended-use statement adopted. 4. SEIS/EIS advance assurance submitted (say "applied for"). 5. At least one signed services engagement (evidence the founder can sell). 6. Data room v1 and one-pager ready; advisory board of at least two named advisers who have consented to be named.
Milestones that make the raise easy (target, not gate)	Three signed services engagements (£15k–£30k contracted); one UK maternity service in discovery conversations towards an unpaid design partnership; one grant bid submitted; a published, sourced report.
Investor type	SEIS/EIS angels (health-tech, NHS clinical/digital leaders, women's-health, Nigerian diaspora professionals in the UK); angel syndicates (GC Angels, NorthInvest, others); one or two pre-seed funds as participants (Ada Ventures, Octopus first-cheque, o2h) if a lead emerges; university seed funds only if a research link exists.
Investor thesis	"A founder-led company that funds itself from services while building, with safety artefacts and honest evidence, a non-diagnostic maternal care-coordination platform for the NHS and low-connectivity settings — in a category where the incumbents are records systems, free charity apps or donor-funded messaging, and none owns coordination and handover."
What the pre-seed buys the investor	The conversion of MaternaLink from a concept into a licensable asset with artefacts : audited codebase owned by the company, safety case, DPIA, DTAC readiness, an MVP, a design partner and a service-evaluation protocol. These are the seed-round gates, so the pre-seed is priced on reaching them.
Dilution (ESTIMATE)	11.1% (£250k at £2.0m pre) to 16.7% (£500k at £2.5m pre); 13.8% for £400k at £2.5m pre in the illustrative table.

Element	Position
Alternatives if it does not close by month 6	Cut founder draw; defer the product lead; run on services (Conservative path); ASA bridge from 2–3 committed angels; Start Up Loan (personal); prioritise an Innovate UK Smart bid; re-open in month 9–12 with more evidence. Rule from 08 §4.1: the product-lead hire must not precede the money.

3.2 Seed — £1.5m to £3m (PROPOSED; Base £2.0m in month 20, i.e. August 2028; High-growth £3.0m in month 14)

Element	Position
Target	£1.5m (floor that funds 18 months of the Base build) / £2.0m (Base) / £3.0m (High-growth, only if the pilot evidence and pipeline justify it). EIS-eligible in part (TO VALIDATE).
Instrument	Priced round; first preference share class acceptable if the lead requires (1× non-participating; no ratchets); pool top-up to ~10% post-money is a normal ask — model it.
Valuation (ESTIMATE, placeholder)	£5m–£8m pre-money; £6m used in the illustrative cap table. Priced on evidence and pipeline, not on FY2028 revenue (~£488k in Base, 59% services).
Use of funds (Base £2.0m over ~24 months, ESTIMATE)	Engineering team: Developer 2, BD lead, programme manager (Nigeria), clinical lead (~£560k) · Implementation contractors and technology at scale (~£500k) · v1.1–v2.2 build: WhatsApp/USSD/voice channels, Yoruba/Hausa/Igbo/Pidgin content, DHIS2 export, FHIR read facade, EPR export interfaces (~£250k) · UK feasibility study (2 sites) and multi-site formal evaluation; Nigeria pilot (1–2 LGAs, 500–2,000 women) with ethics approvals (~£300k) · Compliance: DSPT, Cyber Essentials Plus, DTAC assessment, NDPA registration, device-qualification decision (G8) (~£90k) · Marketing, content and the second e-learning tranche (~£150k) · Buffer (~£150k). Non-people opex in 08 rises to ~£76k/month by FY2029 — that is this money being deployed.
Milestones that must be TRUE before opening the raise	1. MVP built, penetration-tested and in use in a registered UK service evaluation (n ≈ 50–150) under DCB0160 with a trust Clinical Safety Officer — or, if the UK slips, a Nigeria NGO cohort with NHREC-approved evaluation. 2. One paid pilot signed (Base: month 13, £75k/yr ESTIMATE; private maternity, NGO programme or NHS trust). 3. Safety-artefact pack complete: intended-use, DCB0129 hazard log, DPIA, DTAC self-assessment, Cyber Essentials. 4. Team of three or more (founder, product lead/CTO, developer) with the clinical lead at least contracted. 5. Services revenue at run-rate ≥ £15k/month with two retainers (the cash layer proves itself). 6. A documented Nigeria path: signed implementing-partner agreement or state MoU in Vytalix's name , and the Ekiti go/no-go decision minuted. 7. Clean cap table: IP settlement vested and undisputed; pre-seed shares issued with SEIS/EIS compliance statements filed.
Investor type	Impact and health-tech seed funds as lead (AlbionVC health, Ada Ventures, Nina Capital, Goddess Gaia Ventures, Octopus Ventures health, Cambridge Enterprise if research link); Africa-focused funds as co-investors (TLcom, Ventures Platform, Founders Factory Africa, Villgro Africa); Global Innovation Fund and Grand Challenges Canada transition-to-scale as non-dilutive complements; pre-seed angels following.
Investor thesis	"Evidence exists: a UK service evaluation, a paid pilot and a Nigerian cohort show the coordination layer is feasible, acceptable and safe. The seed scales the evidence programme to a multi-site formal evaluation and converts pilots to licences in a UK market of ~120 trusts (ESTIMATE) and an African programme market priced per woman — with a services business that already pays the overhead."
Dilution (ESTIMATE)	20% (£1.5m at £6m pre) to 33% (£3m at £6m pre); 25% for £2.0m at £6m pre in the illustrative table, before pool top-up.
Alternatives	Smaller £750k–£1m extension from pre-seed angels and impact investors (buys 12 months of the Base build without the Nigeria team); SBRI Healthcare Phase 1/2 contract or Grand Challenges Canada seed to fund the evidence programme; slow the cost base (the 08 §6.3 rule: do not scale the MaternaLink cost base until go-live is contractually dated); services-led Conservative path with MaternaLink as a licensed evidence programme.

3.3 Series A — £6m to £12m (PROPOSED; a month 30–42 decision, i.e. mid-2029 to mid-2030; not in the 36-month model)

Element	Position
Target	£6m–£12m, sized at the time from a refreshed model. Not required for solvency in any 08 scenario within 36 months (revision-log item 4).
Valuation (placeholder only)	£15m–£30m pre-money (ESTIMATE; £20m used in the illustrative table). Wholly dependent on ARR, evidence and pipeline at the time; do not cite externally.
Use of funds (ESTIMATE)	Scale to 5–10 UK trusts and 2–3 African state/NGO programmes; in-country teams (Nigeria, Kenya); EPR integrations with the three UK maternity-record vendors; regulatory/QA lead and ISO 13485 if G8/G10 conclude "device"; the v3 research programme (data partnerships, ethics, prospective validation, MHRA SaMD route) — a separate, ring-fenced budget; group restructuring if the hive-down triggers in 02 §1.3 are met.

Element	Position
Milestones that must be TRUE before opening the raise	1. MaternaLink ARR in the £0.4m–£0.9m range (£0.4 \$23 High-growth scenario) or ≥ 3 UK sites and one state-scale programme (10,000+ women) on multi-year contracts. 2. Multi-site formal evaluation reporting process outcomes ("associated with" language). 3. Net revenue retention and pilot-to-licence conversion demonstrated on at least two conversions. 4. Full-time team of ~10 with clinical, regulatory and commercial leads. 5. Africa willingness-to-pay validated at ≥ £6/woman/year or a donor-funded model that covers the fixed team (the "single largest long-run unknown", £8 \$7). 6. Clean regulatory position: documented device-qualification decision; DTAC passed at a trust; NDPA compliance.
Investor type	Health-tech and impact growth funds (UK and pan-European); DFIs for the Africa programme; a strategic (EPR vendor, pharma women's health, telecom/insurer) as a minority participant with no exclusivity.
Investor thesis	"A vendor-neutral coordination layer with evidence and contracts in two health systems, an integration footprint with the UK maternity-record oligopoly, a repeatable partner-first Africa model, and an optional research track towards validated risk insight."
Dilution (ESTIMATE)	23% (£6m at £20m pre) to 38% (£12m at £20m pre); 28.6% for £8m at £20m pre in the illustrative table.
Alternatives	Stay at seed-scale and grow on revenue (the model's High-growth scenario is cash-generative from month 31); strategic partnership/channel deal instead of equity; regional exit (\$ 10_INVESTOR_PROPOSITION exit scenarios); venture debt against contracted ARR.

4. Illustrative cap table (ESTIMATE / PROPOSED — for planning conversations only; not an offer)

Basis: 10,000,000 ordinary shares at incorporation, 100% founder (@ \$2.2). MaternaLink IP settlement 10% (mid-point of the 8–12% PROPOSED range, reverse-vesting 4 years, 12-month cliff). Option pool 12% post-pool (advisors 0.25–0.5% each × 5 ≈ 2% and the CTO/product lead 4–6% are **allocated from within the pool**, so they are shown as sub-lines, not additional dilution). Pre-money placeholders £2.5m / £6m / £20m. No pool top-ups shown (a seed top-up to ~10% post-money would cost all pre-money holders ~2 points pro rata). Percentages fully diluted, rounded to 0.1.

Holder	Incorporation	After IP settlement	After pool	After pre-seed (£400k @ £2.5m pre)	After seed (£2.0m @ £6m pre)	After Series A (£8m @ £20m pre)
Founder (Kelvin Williams — display name ASSUMPTION)	100.0%	90.0%	79.2%	68.3%	51.2%	36.6%
MaternaLink IP counterparty (David Agunede, vesting; subject to IP audit and executed deed)	—	10.0%	8.8%	7.6%	5.7%	4.1%
Option pool (EMI, unallocated at creation)	—	—	12.0%	10.3%	7.8%	5.5%
of which advisors ($5 \times 0.25\text{--}0.5\%$, from pool)	—	—	($\approx 2.0\%$)	($\approx 1.7\%$)	($\approx 1.3\%$)	($\approx 0.9\%$)
of which CTO/product lead and early hires (from pool)	—	—	($\approx 7\text{--}9\%$)	($\approx 6\text{--}8\%$)	($\approx 4.5\text{--}6\%$)	($\approx 3\text{--}4\%$)
Pre-seed investors (SEIS £250k + EIS £150k)	—	—	—	13.8%	10.3%	7.4%
Seed investors	—	—	—	—	25.0%	17.9%
Series A investors	—	—	—	—	—	28.6%
Total	100%	100%	100%	100%	100%	100%
Post-money (ESTIMATE)	—	—	—	£2.9m	£8.0m	£28.0m
Cumulative equity raised	—	—	—	£0.4m	£2.4m	£10.4m

Reading the table.

- The founder remains the largest single holder and above 50% through the seed in this illustration; a founder below ~35% after Series A is normal for venture-backed companies and is the reason the Series A is a *decision*, not an assumption.
- The IP counterparty's stake is meaningful (worth ~£220k at the pre-seed post-money, ~£1.1m at Series A post-money, on paper and unvested) — which is the argument for assignment-for-equity over a royalty licence.

- Sensitivity: every £500k of pre-money at a £400k raise moves pre-seed investor ownership by ~2 points; a £250k SEIS-only round at £2.0m pre gives investors 11.1% (02 \$2.2). Conservative variant (£250k @ £2.0m): founder 70.4%, IP 7.8%, pool 10.7%, investors 11.1%.
- A licence rather than assignment (02 §3 Option 2) would replace the 10% with 2–5% plus a royalty, and investors would price the valuation down or require conversion to assignment as a condition — the table assumes assignment.

5. Raise-readiness checklist (the data room in one page)

Status today: **0 of 32 complete** (VERIFIED absence, FACTS_BASE \$1). Target: all Section A items before any pre-seed outreach; Sections A–C before the seed.

A. Entity and ownership (pre-seed gate)

- ☐ Certificate of incorporation; articles; registers; PSC register; Companies House filings current
- ☐ Founder IP assignment to the company (all pre-incorporation work)
- ☐ MaternaLink IP assignment deed (or exclusive licence + option) executed; IP audit report; code in a Vytalix-controlled repository; "Maternal Link" name and domain transferred
- ☐ Shareholders' agreement; vesting schedules; option-pool resolution (EMI valuation agreed with HMRC when granted — TO VALIDATE)
- ☐ Cap table (fully diluted) matching the registers exactly
- ☐ SEIS/EIS advance assurance application submitted (copy in data room) — later, the HMRC response
- ☐ Bank account; accountant engaged; bookkeeping live; VAT position decided
- ☐ Insurance: professional indemnity, public liability, cyber (certificates)
- ☐ ICO registration; Cyber Essentials (Plus by seed)
- ☐ Trademark applications filed (UK classes 9, 41, 42, 44); name-clearance opinion on file
- ☐ Board minutes from first meeting; Kemek Enterprise conflict declared and minuted

B. Product, clinical and regulatory (pre-seed: started; seed: complete)

- ☐ v1 intended-use statement (non-diagnostic) adopted; 2025 claims retired (with a dated note of retirement)
- ☐ Prototype audit: authorship, licences, security, confirmation no real patient data
- ☐ Clinical Safety Officer contracted; DCB0129 hazard log opened; safety case v1 (seed)
- ☐ DPIA v1; DSPT; DTAC self-assessment (seed)
- ☐ Device-qualification memo (why v1 is not a medical device; what would change that)
- ☐ Product roadmap with gates (/product/MATERNALINK_ROADMAP.md) and the MVP spec
- ☐ Architecture and data-flow diagrams incl. Nigeria hosting/NDPA position

C. Commercial evidence (pre-seed: 1–3 items; seed: all)

- ☐ Signed services engagements (SOWs) and invoices paid; client references with consent
- ☐ Pipeline export from the CRM with stage definitions
- ☐ UK design-partner letter (unpaid) → service-evaluation registration → paid pilot contract
- ☐ Nigeria implementing-partner agreement or MoU in Vytalix's name; Ekiti decision minuted; mDoc complementarity note
- ☐ Discovery-interview summaries (15 UK, 10 Nigeria) with consent
- ☐ Pricing evidence: willingness-to-pay notes per segment; cost-to-serve model

D. Team and governance

- ☐ Founder CV with verifiable facts; time-allocation statement (Vytalix vs Kemek)
- ☐ Advisory board letters (named advisers, consented, compensation stated)
- ☐ Product lead/CTO offer or contract; clinical lead engagement
- ☐ Hiring plan tied to funding milestones

E. Financials and the ask

- [] Financial model workbook (/finance/Vytalix_Financial_Model_3yr.xlsx) with actuals-vs-model from month 2
- [] Use-of-funds table and milestone plan for the round; runway calculation
- [] Term-sheet expectations (one page): instrument, pre-money range, investor rights offered
- [] Investor deck, one-pager, 2-minute script, 15-question answer sheet (/docs/10 , /assets/)

6. Raise-killers — what stops a round dead, and the fix

#	Raise-killer	How an investor finds it	Damage	Fix (owner: Founder unless stated)	Fixed by
1	Undocumented MaternaLink IP — David Agunede signs as "Founder & CEO, MaternaLink"; no assignment, licence or agreement (VERIFIED gap)	One question: "Who owns the code?" Then the email signature block in the Ekiti proposal.	Immediate stop; some investors will not return	Standstill letter (week 1); heads of terms (day 7); IP audit; executed deed (day 30, hard stop day 60); fallback clean-room rebuild (02 \$3; 18 gate G-B)	Day 30–60
2	Unvalidated clinical claims — "predicts pre-eclampsia, sepsis, haemorrhage, thromboembolism"; "reduces maternal mortality"; "expand to multiple NHS trusts"	Any copy of the 2025 deck; a Google search; the Vercel prototype's own text	Regulatory (SaMD without pathway), reputational, and a diligence failure on founder judgement	Retire the deck externally (day 3); adopt v1 intended-use; move AI to a v3 research track; rewrite prototype copy; date-stamp the retirement	Day 3
3	No legal entity	Companies House search returns nothing for "Vytalix"	Cannot issue shares, obtain SEIS/EIS assurance, contract, bank or insure	Name clearance → incorporate (E&W) → bank → accountant → advance assurance (18 gate G-A)	Day 12
4	The Ekiti pricing — £300/woman/year; £4.5m / £9m / £36m phases; in a state with a funded incumbent (mDoc / MSD for Mothers, VERIFIED)	The proposal circulates; any investor with African health-tech exposure knows PROMPTS is under \$1/mother	Destroys pricing credibility and signals no market research	Never restate; re-base to £3–£12/woman/year Africa and £8–£40 UK (08 \$2.3); present the Ekiti decision (partner / move state / stop) as a minuted gate (18 G-D)	Day 21 (re-base); Day 60 (decision)
5	Name and trademark conflict (VITALIX LTD ×2, Vytalix Inc., Vytalize Health)	Trademark search in diligence	Rebrand cost after investment; possible opposition	Knock-out searches; decision; filings (00 problem 4)	Day 5–45
6	Founder focus split (Kemek Enterprise)	LinkedIn; Companies House officer search	"Part-time founder" discount or pass	Written time-allocation rule (≥ 70% Vytalix); declared conflict; board minute	Day 14
7	Prototype of unknown provenance or containing real patient data	Code review; "has anyone entered real data?"	Data-protection breach exposure; unusable asset	Audit; move to Vytalix repo; confirm/clean data; document	Day 30
8	Solo founder with no clinical, technical or regulatory co-leadership	The team slide	Pre-seed passes; seed impossible	Advisory board (two named by day 60); CSO contracted; product lead from pre-seed funds	Day 60 → month 7
9	Services business dressed as venture, or venture dressed as services	The revenue mix (59% services in Base)	Wrong investor, wrong terms	Tell the two-layer story explicitly; price the equity on product milestones (08 \$7 #3)	Now (this document)
10	SEIS/EIS refused or not applied for	"Do you have advance assurance?"	Angel pool shrinks by most of its size	Apply month 1–2 with substance framed around R&D; if refused, restructure or pivot to non-relief investors	Month 2–3

7. Non-dilutive calendar (PROPOSED; every call and amount TO VALIDATE against the live competition pages)

When	Bid	Amount (ESTIMATE)	Pre-conditions	Expected value logic
Month 3–6 (2027 Q1–Q2)	Innovate UK Smart (or the current equivalent open competition) — "offline-first, safety-by-design maternal care-coordination" feasibility/R&D	£50k–£150k grant, 30–55% match	Incorporated; product lead identified; design-partner conversation	15–20% success → EV £10k–£25k; reusable bid material
Month 6–12	SBRI Healthcare — only if a competition theme fits maternity, inequalities or coordination	~£100k Phase 1 (100% funded, TO VALIDATE)	NHS problem-owner letter; DTAC readiness plan	Themed; do not force-fit
Month 9–12	Foundation / trust bid (health inequalities, Black maternal health co-design)	£20k–£75k	Community partner; content plan	Credibility as much as cash
Month 12–18	NIHR i4i Connect (SME) — only with an NHS partner and a clinical-benefit hypothesis for v2 monitoring support	£50k–£150k (TO VALIDATE)	Design partner signed; CSO; safety case	Year-2 item; not in the year-1 plan
Month 12–24	Grand Challenges Canada (seed or transition-to-scale) — Nigeria evidence programme	CAD 100k–1m (TO VALIDATE)	Nigerian implementing partner; NHREC pathway	Aligns with the Nigeria pilot in MATERNALINK_ROADMAP
Month 6–9 (conversation), year 2 (bid)	MSD for Mothers — complementarity with mDoc first	Programme grant (TO VALIDATE)	Ekiti decision; partner-first model	Relationship > bid
Continuous	R&D tax relief (accountant); Start Up Loan if founder chooses (personal liability)	Upside; £25k max personal (TO VALIDATE)	Accounts; payroll	Not modelled; do not depend on

Model discipline: 08 \$6.4 — grants change closing cash, never the peak-need calculation. Write ≥ 4 bids in the first 12 months; 0/4 → remove grants from the model.

8. Investor-process rules (operating the raise)

- 1. Pre-conditions before any outreach** (/investors/OUTREACH_ENGINE.md): incorporated; IP heads of terms signed; 2025 claims retired; one-pager and data room v1 ready; SEIS/EIS advance assurance submitted.
- 2. Sequence:** warm angels and syndicates first (Call Lists 1–2), funds second, grants in parallel. Target for the pre-seed: 25–40 first conversations, 10 meetings, 3–5 committed cheques to anchor, then fill.
- 3. Language discipline in every conversation:** "MaternaLink (in development)"; "applied for" not "approved"; "proposal" not "partner"; "prototype" not "platform"; scenario numbers labelled as scenarios.
- 4. One CRM, one pipeline, 18 stages** (/investors/CRM_PIPELINE.md); log every touch; weekly investor review in the Friday cadence; Kemek Enterprise contacts never imported.
- 5. Term discipline:** ordinary shares at pre-seed; no anti-dilution ratchets; no board seat for a £25k cheque (observer at most for the largest angel); information rights quarterly; drag/tag standard; pool top-ups modelled before agreeing.
- 6. Close discipline:** a round is announced only when the shares are issued and the SEIS/EIS compliance statements are filed. Internally, "committed" ≠ "signed" ≠ "received".
- 7. Stop rules:** pre-seed not closed by month 6 → Conservative path and re-open in month 9–12; seed gates not TRUE by month 18 → extension from existing investors or slow the cost base; never hire ahead of cash.

Priorities / Risks / Next actions

Priorities

1. Close the four raise-killers (IP, claims, entity, Ekiti pricing) in the first 60 days — nothing else in this document works until they are closed.
2. Submit SEIS/EIS advance assurance in month 1–2 with the application framed around the R&D product programme; structure the Base pre-seed as SEIS £250k then EIS £150k (limits TO VALIDATE).

3. Open the pre-seed only when the \$3.1 gates are TRUE; target close month 4 (Base) and never hire the product lead ahead of the money.
4. Run two grant bids in year 1 (Innovate UK Smart; SBRI if a theme fits) as upside, and start the MSD for Mothers / mDoc complementarity conversation by month 9.
5. Price every round on milestones (artefacts, design partner, paid pilot, evidence), and keep the illustrative cap table as the written dilution plan.

Risks

- SEIS/EIS refused or delayed because the trade looks like consultancy or because the IP route involves royalties → restructure the application, choose assignment over licence, and prepare a non-relief investor list.
- Pre-seed slips past month 6 → founder draw cut, product lead deferred, ASA bridge; company collapses toward the Conservative scenario.
- IP dispute or no agreement by day 60 → clean-room rebuild; MaternaLink cannot be described as owned until then.
- Seed gates depend on third parties (an NHS evaluation site, a Nigerian partner, ethics timelines) → fallback sites and an extension round from existing investors.
- Valuation placeholders (£2.5m / £6m / £20m) are unvalidated → test with three friendly investors before any term sheet.

Next actions (owner: Fundraising Director with Founder)

- Week 1: standstill letter to David Agunede; retire the 2025 deck; name-clearance search; accountant briefed on SEIS/EIS.
- Week 2: incorporate; open bank; instruct solicitor on the IP deed and shareholders' agreement; draft the advance-assurance pack.
- Week 4: IP deed executed; data room v1 (Section A of \$5) assembled; advance assurance submitted; update `02_GROUP_STRUCTURE` §2.2 and `/assets/BUSINESS_PLAN.md` to the £250k–£500k pre-seed sizing.
- Week 6–8: valuation sounding with three investors; Call Lists 1–2 outreach begins (pre-conditions met); Innovate UK Smart bid outline.
- Month 4: pre-seed target close; re-run the model with actuals; re-label this document's ESTIMATES that have become KNOWN.